
CPT 92229 Reimbursement Brief for Federally Qualified Health Centers

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Date: May 14, 2026

Version: v2b (verification-checked, see source list at end)

Audience: FQHC billing offices, CFOs, compliance reviewers, and medical directors evaluating autonomous AI diabetic retinopathy screening for their patient panels

How to Use This Brief

This is a partner-ready document. Proactive FQHC is sharing it with member FQHCs evaluating CPT 92229 deployment. It is written for your billing office, CFO, or compliance reviewer, not for Proactive itself.

Wherever you see "your FQHC" in this document, read it as your specific community health center being evaluated. Specific billing rates, APM eligibility, and MCO contract terms are site-specific and must be confirmed by your own billing team with your MAC and state Medicaid office.

This brief is informational. Nothing in it is a contractual commitment, and final reimbursement determinations rest with your MAC, fiscal intermediary, state Medicaid office, and MCO contracts.

Scope and Caveat

This brief covers CY 2026 reimbursement guidance for CPT 92229 (autonomous AI diabetic retinopathy screening) as it applies to FQHCs. Sources include CMS, California DHCS, Oregon OHA, Washington HCA, and NACHC.

Each partner FQHC must confirm final numbers with its own MAC and state Medicaid office before treating any figure as a commitment. FQHC PPS encounter rates are facility-specific. Managed Medicaid contracts vary plan-by-plan. APM eligibility and weighting differ by state. Nothing in this brief overrides your fiscal intermediary's guidance.

Short Answer

For FQHCs, CPT 92229 does not generate a separately payable line in any of the reimbursement lanes covered here. It is absorbed into the all-inclusive encounter rate (FQHC PPS), the per-member-per-month

payment (APM), or the MCO contract rate. The financial case for 92229 at an FQHC is not a per-screen fee. It is three things stacked together:

1. **Encounter capture.** Every diabetic patient who would have been referred out for retinal screening is now an in-clinic encounter that bills at your FQHC PPS rate (roughly \$208 Medicare, \$220 to \$440 California Medi-Cal, facility-specific in every state).
2. **HEDIS Eye Exam for Patients with Diabetes (EED) performance.** Drives MCO pay-for-performance, commonly 1 to 2 percent of plan premium or contract value.
3. **APM participation eligibility.** In states with APM options (California CalAIM, Oregon, Washington), sustained quality performance is a participation condition, not a bonus. Dropping below threshold reverts your FQHC to traditional PPS billing, which is a structural revenue change.

The rest of this brief works through each lane in detail.

What Counts as a Qualifying FQHC Encounter

This is the federal rule that governs every FQHC conversation about 92229, worth naming up front because it changes how a "dedicated DR screening visit" plays out.

Per **42 CFR 405.2463**, a qualifying FQHC visit requires a face-to-face encounter between the patient and an FQHC-recognized practitioner, for a medical, mental health, or dental purpose. State Medicaid FQHC PPS rules are generally aligned or stricter.

Qualifying practitioners under Medicare FQHC PPS per 42 CFR 405.2463: physician (MD or DO), nurse practitioner, physician assistant, certified nurse midwife, clinical psychologist, and clinical social worker. (Medicare FQHC PPS does not generally cover dental services through these qualifying-visit billing rules; state Medicaid FQHC programs may include dental encounters separately.)

What does NOT qualify on its own:

- A screening or diagnostic service delivered by a medical assistant or ophthalmic tech under standing order, without a face-to-face with a qualifying practitioner. 92229 delivered stand-alone by non-practitioner staff does not trigger a PPS encounter.
- "Incident-to" nursing services, lab draws, or imaging that occur without a practitioner face-to-face.

What DOES qualify:

- A 92229 delivered during an existing qualifying visit (AWV, chronic care follow-up, problem visit, diabetes management). The screening is bundled into the encounter; the encounter still pays full PPS.
- A "dedicated" visit that pairs 92229 with a brief qualifying-practitioner touch (NP, PA, or MD sees the patient for a diabetic status check, med reconciliation, AWV, or chronic care management interaction). The encounter is the provider visit, not the screening.

Source: [42 CFR 405.2463](#), *FQHC qualifying visit definition*.

Lane 1: Medicare PPS for FQHCs

CY 2026 base encounter rate: \$207.72 per PPS-eligible visit (CY 2025 was \$202.65, a 2.5% increase).

Adjustments:

- Geographic Adjustment Factor (GAF) adjusts the base rate by locality.
- 34.16% increase for new patients, Initial Preventive Physical Examination (IPPE), or Annual Wellness Visit (AWV). Boosted rate: $\$207.72 \times 1.3416 =$ approximately **\$278.68**.
- Medicare pays 80% of the lesser of charges or PPS rate, after 20% patient coinsurance.

Is 92229 separately payable? No. Under FQHC PPS, all medical services on the date of a qualifying visit roll into the single encounter payment. Diabetes Self-Management Training (DSMT), Medical Nutrition Therapy (MNT), and a defined list of preventive services are recognized as separate billable encounters under FQHC PPS; certain mental health visits on the same day as a medical visit can also generate a separate encounter. Diagnostic imaging codes, including 92229, are not on the separately-payable list and are bundled into the qualifying-visit encounter.

CMS has not issued 92229-specific bundling guidance by code name; the bundling conclusion above flows from the general FQHC PPS payment principle that procedures performed the same day as a qualifying encounter are not separately billable. Some state Medicaid programs do carve specific services out of PPS as separately payable (vision services in some states); your FQHC's billing office should confirm any state-specific carve-out before treating 92229 as separately payable in any lane.

What this means for your FQHC: delivering 92229 during a qualifying visit does not generate incremental Medicare PPS revenue. It does, however, capture an encounter your clinic would otherwise lose to an external referral, and it closes the HEDIS Eye Exam for Patients with Diabetes (EED) gap for the patient.

Sources: [CMS FQHC PPS](#), [CY 2026 FQHC PPS Payment Rate Update \(R13506BP\)](#), [NACHC FQHC Payment Guide 2025](#).

Lane 2: Medicaid PPS for FQHCs (State-Specific)

Medicaid FQHC PPS rates are facility-specific and set by state through cost-report-based methodology. 92229 is bundled in every state PPS lane covered in this brief. The encounter rate is the revenue unit.

California (Medi-Cal)

- Facility-specific PPS rates, published by DHCS.
- Verified 2025 examples cluster around \$220 to \$440 per encounter (Highland / Alameda County \$220.22, Contra Costa County Health Services \$440.40, SF General \$288.35, Newark \$310.28).
- Proposition 35 set a Medi-Cal payment floor for non-FQHC providers at 87.5% of Medicare rates for primary care, maternity, and certain mental health services beginning January 2024, with additional Prop 35-funded rate increases taking effect January 1, 2026. This applies to non-FQHC providers and is relevant context for MCO negotiations and downstream referral economics, not to your FQHC's encounter rate directly.

Oregon (Oregon Health Plan)

- Facility-specific PPS set at 100% of average FQHC costs at base year, indexed annually by the Medicare Economic Index (MEI).
- APM option is separately described in Lane 4 below.

Washington (Apple Health)

- Facility-specific encounter rates under PPS.
- APM option described in Lane 4.

What this means for your FQHC: the relevant number is your own facility-specific encounter rate in each state where you operate. Your billing team should pull the current published rate (or the last rate letter from the state) rather than relying on national averages. Rates can move 2x across facilities even within the same state.

Sources: [Alameda Health System FQHC Finances \(CA PPS examples\)](#), [DHCS FQHC Rate Setting Instructions](#), [Washington HCA FQHC Billing Guide](#), [Oregon APM Program FAQs](#).

Lane 3: Managed Medicaid (MCO)

How it works: FQHCs contract with Medicaid Managed Care Organizations at plan-negotiated rates. The MCO pays the contract rate. The state Medicaid agency then pays a wrap-around payment equal to the difference between the MCO payment and your FQHC PPS encounter rate, ensuring you are made whole at 100% of PPS.

Is 92229 separately payable? No. Same bundling as Lane 2 applies to the underlying encounter. However, 92229 delivery commonly maps to HEDIS EED closure, which plays into MCO pay-for-performance.

Pay-for-performance examples (contract-specific, directional only)

- **Ohio Medicaid:** the original P4P program (2013) capped bonuses at 1% of total annual premium with 100-110% multipliers. As of 2024-2025, Ohio uses a **Quality Withhold methodology** rather than a bonus-pool structure. MCOs that hit quality targets earn back withheld payment.
- **Nevada Medicaid:** monetary bonuses for MCOs hitting the 90th percentile HEDIS or 10% year-over-year improvement.
- **Typical MCO-to-provider language:** 1 to 2 percent of contract value at risk tied to HEDIS performance, with quality-tier multipliers above that.

Do not treat any of these as a quote. They are directional. The only numbers that matter are your specific MCO contract terms.

Sources: [Noridian FQHC Medicare Advantage Wrap-Around Payment](#), [Nevada State Plan Pay for Medical Care](#).

Lane 4: Capitated / PMPM (State APM Programs)

This is the lane that will grow fastest over the next 24 months in West Coast states. Every West Coast state now offers an FQHC-specific APM that replaces PPS encounter billing with a per-member-per-month capitation.

California CalAIM FQHC APM

- Implementation waves: first cohort July 1, 2024. Second expansion effective January 1, 2026.
- PMPM capitation replaces PPS encounter rate and wrap payments. FQHCs do not bill 0521 T1015 SE wrap claims for APM enrollees.
- DHCS guarantees a PPS-floor: annual reconciliation ensures FQHCs receive at least what they would have under traditional PPS.
- **Continued participation is conditional on quality metric performance.** Tied to the Medi-Cal Managed Care Accountability Set (MCAS). MCAS Reporting Year 2026 includes Glycemic Status Assessment for Patients with Diabetes (GSD) as a chronic disease measure (replacing the prior CDC-HbA1c Poor Control measure); the Population Specific Plans (PSP) subset of MCAS RY 2026 contains 13 measures. EED is a HEDIS diabetes measure that maps to MCAS chronic disease reporting; specific APM scorecard weight varies by plan.
- FQHCs must maintain at least 70% of historical PPS-eligible utilization to stay in the program.
- The CY 2026 Cohort 2 application deadline was December 16, 2024. Application deadlines for future cohorts are set by DHCS annually; confirm the current deadline directly with DHCS before applying.

Oregon APM

- Prospective PMPM replaces FFS encounter payment for all services except dental, mental health, and obstetric care.
- 20 of Oregon's 33 Community Health Centers now participate, plus two Rural Health Clinics.
- Federal floor: APM must reimburse FQHCs at least as much as they would have received under PPS.
- Portions of payment adjusted based on performance on annually-refreshed quality measures.

Washington APM (APM 3 and APM 4)

- Program launched July 1, 2017 with 16 of Washington's 27 FQHCs participating. Current participation count should be confirmed directly with the Washington Health Care Authority.
- PMPM calculated from the FQHC's APM encounter rate times billable-encounter volume, divided by total Medicaid managed-care months, trended annually by MEI.
- PMPM rate is protected from going below the encounter-based equivalent.
- Portion of payment adjusted against five performance measures, refreshed annually.

Is 92229 separately payable under any APM? No. It is structurally embedded in the PMPM, and the value shows up as quality measure performance that protects continued APM participation. A missed threshold can push your FQHC back into PPS billing, which is a material revenue-model change, not a missed bonus.

Sources: *DHCS FQHC APM Program Guide*, *MCAS Reporting Year 2026, CY 2026 FQHC APM Application Instructions*, *Oregon APM Program FAQs*, *NACHC: The FQHC Alternative Payment Methodology*, *Health Management Associates: Washington FQHC APM*.

Lane 5: Medicare Advantage at FQHC

- Your FQHC bills the MA plan at the contracted MA rate.
- Medicare pays a supplemental wrap so your FQHC nets 100% of Medicare FQHC PPS equivalent.
- 92229 bundled, same logic as Lane 1.
- FQHCs submit two claims (one to MA, one to Medicare, TOB 77X, rev code 0519).

Source: *Noridian FQHC Medicare Advantage Wrap-Around Payment*.

Summary Table for FQHC Reimbursement Lanes

REIMBURSEMENT LANE	RATE UNIT	APPROX. AMOUNT (CY 2026)	IS 92229 SEPARATELY PAYABLE?
1. Medicare FQHC PPS	Per-encounter	\$207.72 base, GAF-adjusted	No, bundled
2. Medicaid FQHC PPS (California example)	Per-encounter	\$220 to \$440 facility-specific	No, bundled
3. Managed Medicaid	Contract rate + state wrap to PPS floor	Varies by plan	No, bundled; drives HEDIS bonus
4. CalAIM / Oregon / Washington APM	PMPM capitation	Facility-specific, quality-gated	No, quality-metric-linked
5. Medicare Advantage at FQHC	Per-encounter (with wrap)	~\$207.72 equivalent	No, bundled

For context only, non-FQHC Medicare FFS providers receive a CMS Physician Fee Schedule rate that trended down from \$47.06 in CY 2022 to \$40.28 in CY 2024, with CY 2026 locality-adjusted rates published in the CMS PFS Look-up Tool. This per-screen number is the figure commonly quoted online and applies to private practices and non-FQHC facilities. **It does not map to FQHC economics, where the same screening is bundled into the encounter rate.** Your FQHC's billing office should pull the current locality rate for your zip code directly from the CMS PFS Look-up Tool if non-FQHC comparison is needed.

The Three-Part Value Story for an FQHC Deploying AEyeDS

Since 92229 does not generate a per-screen fee at an FQHC, the financial case should be framed in these three stacked components:

1. Encounter Capture

- Every diabetic patient on your panel who is currently referred out for retinal screening is an encounter your FQHC does not bill.
- At California Medi-Cal PPS rates of \$220 to \$440 per encounter, one avoided referral covers a meaningful share of program cost.
- 92229 delivered in-clinic is an encounter that would otherwise go to an external ophthalmologist or retina specialist, often with meaningful no-show and completion rates.

2. HEDIS EED Quality Performance

- HEDIS Eye Exam for Patients with Diabetes is the relevant national measure for adults aged 18 to 75 with diabetes.
- EED drives MCO pay-for-performance bonuses, typically in the 1 to 2 percent of plan premium range, and is built into state accountability sets including California's MCAS.
- Point-of-care AI screening typically moves compliance meaningfully because it removes the referral-to-completion gap.

3. APM Participation Protection

- For FQHCs in APM states (California CalAIM, Oregon, Washington): quality performance is a condition of staying in the APM. Falling below quality thresholds reverts your FQHC to traditional PPS billing, which is a structural change in revenue model, not just a missed bonus.
- EED closure is commonly weighted in APM scorecards.
- For FQHCs under MCO contracts in non-APM states: DR screening often maps to a specific P4P quality gate, not just a bonus multiplier.

The AWW Pull-Through Play (the cleanest tactical path)

Given that 92229 is bundled in every FQHC lane, and that a screening-only visit does not trigger a PPS encounter, the workflow most FQHC billing offices will sign off on is what is referred to here as the Annual Wellness Visit (AWV) pull-through play.

The setup:

1. Pull a list of Medicare-aged diabetic patients on your panel who have not had an Annual Wellness Visit in the current benefit year.
2. Proactively outreach that list for AWW scheduling.
3. Deliver 92229 during the AWW, in the same visit.

The math on a single Medicare visit (CY 2026):

COMPONENT	AMOUNT
Base FQHC PPS encounter	\$207.72
AWV / IPPE / new-patient bump (34.16%)	+\$70.96
Total encounter payment	\$278.68
92229 captured in the encounter	bundled, no incremental fee
HEDIS EED closure for the year	yes, one visit closes the measure

Why this works:

- The AWV is a qualifying preventive service under 42 CFR 405.2463, so the encounter is billable.
- The 34.16 percent new-patient, IPPE, AWV bump is explicitly authorized in CMS FQHC PPS guidance.
- 92229 inside the AWV captures the diagnostic screening without requiring a second encounter or a dedicated DR screening day.
- EED closure on the same visit cascades into MCO P4P and APM quality-threshold protection.

Why it beats the "dedicated DR screening visit" framing:

- Dedicated DR screening days without a qualifying practitioner touch do not trigger a PPS encounter (per 42 CFR 405.2463).
- Dedicated DR screening days with a qualifying practitioner touch do work, but they consume scarce NP/PA time per patient and compete with existing visit templates.
- AWV pull-through uses a visit slot you are already running, adds 92229 to it, and gets the AWV bump on top. Less operational change, higher per-visit revenue, same quality-measure closure.

Why this works (operational notes):

- The AWV's Personalized Prevention Plan should include diabetic retinopathy screening for diabetic patients aged 18 to 75. Diabetic retinopathy screening is not a CMS-mandated AWV component, but it fits cleanly into the Personalized Prevention Plan recommendation set for any diabetic patient.
- 92229 inside the AWV closes the HEDIS EED measure for the year. EED is an NCQA administrative-only measure as of Measurement Year 2025.

One important Medicaid clarification:

The 34.16% AWV bump is a Medicare FQHC PPS adjustment. State Medicaid FQHC PPS programs (California Medi-Cal, Oregon Health Plan, Washington Apple Health) do not generally apply a 34.16% bump on top of their facility-specific PPS rate. A Medicaid "AWV-equivalent" visit (such as California's Initial Health Appointment, billed within 120 days of enrollment) triggers a PPS encounter at your clinic's

standard PPS rate without a Medicare-style bump. The bump and 92229 pull-through value proposition is strongest in the Medicare lane.

What your FQHC billing office should confirm:

- AWW eligibility for each targeted diabetic patient. G0438 (Initial AWW) is once per lifetime, after 12 months of Medicare Part B enrollment and not within 11 full months of an IPPE. G0439 (Subsequent AWW) requires 12 full months between AWWs.
- MAC guidance on documenting 92229 as an incident diagnostic during the AWW encounter.
- Any state Medicaid AWW-equivalent visit type that triggers a PPS encounter in your footprint (California's IHA, similar structures in Oregon and Washington), recognizing that no 34.16% bump applies in the Medicaid lane.

Sources: [CY 2026 FQHC PPS Payment Rate Update \(R13506BP\)](#), [42 CFR 405.2463](#), [NACHC FQHC Payment Guide 2025](#).

What Your FQHC Should Confirm Before Deploying

1. **Current Medicare FQHC PPS encounter rate** (with GAF applied) for each site.
2. **Medicaid PPS encounter rate per site**, and APM eligibility in each state where you operate. Pull the most recent rate letter from your state Medicaid office; rates can move year over year.
3. **The specific EED weighting** in your MCO contracts and any APM scorecards your clinic is subject to.
4. **Whether APM participation is currently active** or whether you are on traditional PPS, by state and by site.
5. **Any state-specific exceptions to 92229 bundling**. National FQHC PPS guidance bundles 92229 into the encounter, but some state Medicaid programs publish service-specific carve-outs. Your MAC is authoritative for Medicare; your state Medicaid program is authoritative for Medicaid lanes.

Next Steps

If your FQHC wants to evaluate AEyeDS deployment:

- Contact Proactive FQHC at grants@proactivefqhc.org for partnership and grant-program guidance.
- Contact AEYE Health at mike@aeyehealth.com (Regional Sales Manager, West) for product, deployment, and pilot scoping.
- The AEYE ROI calculator at <https://aeyehealthroi.netlify.app/> is available for initial financial modeling, with a more detailed version linked from Proactive's retinopathy resources page.

Source List

- [CMS FQHC PPS](#)
- [CY 2026 FQHC PPS Payment Rate Update \(R13506BP\)](#)
- [Medicare FQHC PPS FAQs](#)
- [FQHC PPS Specific Payment Codes](#)
- [CMS Billing and Coding: Remote Imaging of the Retina \(A58914\)](#)
- [Noridian FQHC Billing Guide \(JE Part A\)](#)
- [Noridian FQHC Medicare Advantage Wrap-Around Payment](#)
- [NACHC FQHC Payment Guide 2025](#)
- [NACHC: The FQHC Alternative Payment Methodology](#)
- [DHCS FQHC APM Program Guide](#)
- [DHCS Medi-Cal APM FQHC Billing Instructions](#)
- [DHCS CY 2026 FQHC APM Application Instructions](#)
- [DHCS FQHC Rate Setting Instructions](#)
- [MCAS Reporting Year 2026](#)
- [Alameda Health System FQHC Finances \(CA PPS rate examples\)](#)
- [Oregon APM Program FAQs](#)
- [Washington HCA FQHC Billing Guide](#)
- [Health Management Associates: Washington FQHC APM](#)
- [Commonwealth Fund: Perils and Payoffs of APMs for CHCs](#)
- [Digital Diagnostics: CMS Finalizes National Rate for CPT 92229](#)
- [AEYE Health: Billing and Reimbursement for Autonomous Diabetic Eye Exam](#)
- [California Legislative Analyst's Office: Proposition 35 implementation timeline](#)
- [Ohio Medicaid Managed Care Quality Withhold Methodology CY 2024-2025](#)
- [Oregon Primary Care Association APCM page](#)
- [NCQA HEDIS EED Measure description](#)
- [CMS Annual Wellness Visit](#)
- [NACHC IPPE/AWV Reimbursement Tips](#)
- [IHS-JVN November 2025 Teleophthalmology AI Handout \(industry context on AI-referred patient follow-up rates\)](#)

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This brief is directional planning information only, not a quote, not a contract. Confirm all figures with your MAC, fiscal intermediary, IHS Area Office, state Medicaid program, and MCO contract administrator.